

HEALTH CARE LAW & ETHICS: FINAL PROJECT

This project involves a hypothetical scenario where Kinetic Surgical is developing two products. *[Note: this is a completely fictional scenario, and no actual business performed / will perform these specific actions.]* Kinetic Surgical was founded two years ago in St. Paul, Minnesota, with ambitious goals of emerging into the orthopedic rehabilitation market by developing analytics-driven medical devices at a low cost. The company's vision is led by CEO, Buzz Aquinas, who spent 14 years in orthopedic medical device sales, and has seen first-hand how technology can dramatically improve patient recovery after surgery. Buzz is joined by Charlie Thomas, Kinetic Surgical's VP of Sales and Marketing, who similarly has a long history in the medical device sales industry and is ambitiously focused on attracting investors in the medical tech community.

The company's first two product concepts show immense promise. The FlexiStep, an adjustable ankle stabilization brace with built-in motion sensors, is designed to help patients recovering from ankle and foot surgeries regain strength more safely. The second product, the KneeComfort Pro, is a contoured foam positioning support intended for use during physical therapy or post-surgical recovery to help patients maintain proper alignment and reduce pain. While both products are still in the prototype stage, the talk around the office is quickly starting to spread that Kinetic Surgical's designs bear striking similarities to those of two industry giants: Stryker Corporation and BoneFoam, Inc.

Buzz openly admired Stryker's Infinity Total Ankle System, one of the most widely used total ankle replacement implants in the country. Although Kinetic Surgical's FlexiStep is not an implant, the engineering team had closely studied Stryker's publicly available materials on alignment philosophy, load distribution, and component geometry, focusing most on the Infinity System's titanium tibial tray, cobalt-chromium talar dome, and polyethylene insert design. While none of these features were directly copied, internal design notes show diagrams referencing Stryker's component shapes and alignment angles as "industry benchmarks." Concerns have been raised that Stryker's implant portfolio is protected by a combination of design patents, utility patents, and proprietary engineering data, and competitors have historically faced aggressive enforcement actions for products that appear derivative or reverse-engineered. FlexiStep's outer shell geometry and its load-stabilizing frame resembled elements seen in Stryker's marketing materials, and several employees privately questioned whether the similarities were too close for comfort.

The KneeComfort Pro raises similar concerns. BoneFoam, Inc., known for its surgical positioning and rehabilitation products, had built a strong reputation for ergonomic foam devices. These products were backed by a series of patents protecting their distinctive shapes, dimensions, and materials. When Kinetic Surgical's prototype foam support debuted internally, several employees noticed it looked almost identical to BoneFoam's models—down to the contours

that supported the calf, and detachments to create alternate support angles. According to one engineer, some of the early design work was based on reverse-engineering images and product specs publicly available online. Though possibly not illegal in itself, this practice raises red flags for the company's general counsel, who warns that too much overlap can risk an intellectual property dispute.

At a board meeting last week, the Kinetic Surgical leadership team spent time brainstorming ways to successfully gather the clinical and patient data necessary to demonstrate product safety and effectiveness. Charlie was confident from his years working with physicians in the operating rooms and proposed leveraging his professional relationships with physicians using Stryker and BoneFoam products. He stated many had extensive post-operative data from patients, and Kinetic Surgical could benchmark their own products' performance against this data to refine the designs before launch. While in theory this data is comprehensive, reliable, and directly related to the patient population Kinetic Surgical is aiming to help, Buzz questioned the reality of physicians handing over data that is part of their own research or the IP portfolio of their top competitors. He fiercely pushed back and suggested the company should instead build an interactive public website where patients could voluntarily upload their recovery data, including their range of motion, pain levels and therapy progress. In exchange, participants would receive free sample products or early access to Kinetic Surgical's recovery-tracking app. Buzz pressed the leadership team to think about the competitive edge this would give the emerging company, in addition to obtaining the patient data in a timely, low-cost way.

The tension in the board room continued to grow as the general counsel pushed back on both potential ideas. Asking the physicians to hand over patient data might violate HIPAA and the physician's fiduciary duty to maintain patient confidentiality, while the public website proposal carries the risk of patients not fully understanding how their data would be used, and how would the courts interpret the incentives provided to participating patients? Was the company prepared to handle this volume of patient data without risk of privacy breaches or regulatory investigations?

The leadership team quickly realized they were at a crossroads. Their prototypes were functioning well in early tests and top investors were scheduled to come into town next week to continue discussing the upcoming clinical trials. Buzz and Charlie agree to hire some outside consulting teams to identify the legal risks, the interests of affected stakeholders, and the strategic and ethical tradeoffs before making a recommendation on how to proceed.

ASSIGNMENT: Your group's responsibility is to develop recommendations about your assigned product, that address the legal risks, and strategic and ethical considerations raised by the development and marketing of your product.

1. Everyone will skim article on COVID vaccines policies (focus on the tables and examples). This article gives you some background about being creative to address ethical tradeoffs, in developing a strategic solutions to possibly conflicting health care business concerns.
2. Background write-up.
 - a. Legal Concerns.
 - b. Ethical Stakeholders
 - 1) Employees / Managers / Owners
 - 2) Customers (patients and their physicians)
 - 3) Doctors with data / Individuals providing data
3. Identification & Description of Legal, Strategic & Ethical Tradeoffs
4. Written recommendations for your business, with explanations of how these recommendations address legal concerns and ethical tradeoffs. These recommendations may include alternative recommendations.
5. Presentation of these recommendations, noting key handling of tradeoffs.
6. Group members not presenting will need to come up with one question each for an assigned other group.

Groups will be presenting their recommendations during the final exam period (Tuesday December 16, 1:30-3:30). Each group will get a total of about twenty-five (25) minutes to present and answer questions from the class, with no more than fifteen minutes for their presentation.

Group members will also be completing a Qualtrics survey concerning group members' participation. Students will be expected to attend each class session, and if they cannot, need to send me a note describing how they have or will contact other group members to make up for not being present.

Assignment	Due	What turned in, and by how many group members?
Divide up write-up responsibilities	18-Nov	Group turns in responsibility list at end of class
Write-up	1-Dec	Each Group Member
Describe Tradeoffs & Complementarities	5-Dec	Written up by one
Recommendation / Address Tradeoff	10-Dec	Worked on by two team members
Presentation Powerpoint / PDF	12-Dec	One submission for whole group (done by one or two students)
Questions for presenters	15-Dec	One question per person (can submit common group of questions)
Presentation & Question session	16-Dec, 1:30pm	
Qualtrics Survey	18-Dec	

